

1. Module 4: Communications and Marketing Rules for Medicare Advantage and Part D Plans

2. Navigation Instructions

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4. Learning Objectives

- After reviewing "Module 4: Communications and Marketing Rules for Medicare Advantage and Part D Plans" you will be able to explain:
 - What are communications
 - What activities constitute marketing and what materials are marketing materials
 - The special rules for marketing Medicare health plans and Part D plans
 - Rules for making marketing appointments
 - Prohibited marketing and communications practices
 - Permitted promotional and marketing activities
 - The difference between educational and marketing events
 - Rules regarding agent compensation
 - Plan enforcement of the marketing rules and potential penalties

5. Training Roadmap: Module 4

- Applicability of rules concerning marketing and communication
- Marketing Representatives: identification, state licensure, and training
- Key terms and general background information
- Regulation of marketing activities and materials
- Marketing and communications prohibitions and permissible activities

- Rules related to sales/educational events and marketing at individual appointments
- Required and prohibited marketing practices
- Accessing and using certain plan materials
- Plan ratings
- Prohibited practices and marketing prohibitions
- Oversight and enforcement of marketing requirements
- Marketing Representative Compensation

6. Title Page – Applicability of Rules Concerning Marketing and Communications

7. Medicare Marketing and Communications Rules

The Medicare marketing and communications rules apply to the following types of Medicare health plans and Part D plans:

- Medicare Advantage (MA) only plans
- Medicare Advantage Prescription Drug (MA-PD) plans
- Prescription Drug Plans (PDPs)
- Section 1876 Cost plans
- Medicare-Medicaid Plans (MMPs)
 - For MMPs, marketing requirements may be modified by state-specific requirements. Each state in which MMPs are offered has state-specific marketing guidelines and CMS-approved model documents. Those guidelines and documents can be accessed at <https://www.cms.gov/Medicare-Medicaid-Coordination/Medicare-and-Medicaid-Coordination/Medicare-Medicaid-Coordination-Office/FinancialAlignmentInitiative/MMPInformationandGuidance/MMPMarketingInformationandResources.html>

8. Applicability of Medicare Communications and Marketing Rules to Marketing Representatives

Plan marketing representatives are subject to the same requirements related to marketing and communications as the plans. Plans are responsible for ensuring compliance with Medicare rules by their marketing representatives.

Plan marketing representatives include:

- individuals employed by a plan
- individuals or entities under contract to the plan through a direct or downstream contract.
 - This would include brokers and agents (contracting directly with the plan or through an agency or other entity), third-party marketing organizations (TMOs) such as field marketing organizations (FMOs), general agents (GAs), or other marketing contractors).

9. Title Page – Marketing Representatives

10. Requirements to act as Plan Marketing Representatives

- Plan sponsors must contract with or employ marketing representatives that are licensed by the state if they conduct the relevant activities in that state.
- The marketing representatives must be appointed by the plan if required under State law.
- MA and Part D plans are required by law to ensure that all employed and contracted marketing representatives complete training at least annually that includes all content specified by CMS.
 - They also must pass a written test each year that demonstrates thorough familiarity with both the Medicare program and the products they are selling.
 - Agents/brokers marketing only employer/union group plans are not required to be tested. However, plans may choose to require testing.

11. Marketing Representatives – State Licensure Case Study

Agent King is employed by an agency under contract with HealthMax health plan, a Medicare Advantage organization that offers plans in multiple states. The agency maintains a website marketing the Medicare Advantage plans with which it has contracts. Agent King follows up with individuals who request more information about HealthMax via the agency's website and tries to persuade them to enroll in HealthMax. Agent King is a marketing representative of HealthMax. Thus, he must be licensed and appointed in every state in which beneficiaries to whom he markets HealthMax are located. He may not submit enrollment applications or receive commissions for enrollees residing in states in which he is not licensed and appointed.

12. Third Party Marketing Organizations

- Third Party Marketing Organizations (TPMOs) are organizations and individuals, including independent agents and brokers, who are compensated to perform lead generation, marketing, sales, and enrollment related functions as a part of the chain of enrollment (the steps taken by a beneficiary from becoming aware of an MA or Part D plan or plans to making an enrollment decision). TPMOs may directly contract with the plan sponsor, may be a downstream contractor of a Plan Sponsor, or may be a related entity of the Plan Sponsor (referred to as first tier, downstream or related entities or FDRs). However, they may also be entities that are not FDRs but provide services to an MA plan or an MA plan's FDR.
- Independent agents and brokers are those that are not directly employed by a plan and those that are market for more than one plan.
- As detailed in this training, beginning January 1, 2023 a number of new requirements will be applicable to TPMOs in relation to their marketing of 2023 and subsequent year plans beginning October 1, 2022.

13. Title Page – Key Terms and General Background Information

14. Marketing and Communication – In General

- **Communications** are materials and activities that provide information to current and prospective enrollees on Medicare Health Plans and Part D Plans.
- **Marketing** is a subset of communications. Marketing is distinguished from communications-only material and activity based on intent and content.
- Marketing activities and materials are generally subject to a higher degree of regulation and oversight.

15. What is Marketing?

To be considered marketing, the material or activity must include both marketing intent and marketing content.

- **Marketing Intent** – the purpose of marketing activities and materials is to draw a prospective or current enrollee’s attention to a plan or group of plans to influence a beneficiary’s plan choice, including a decision to remain enrolled in his/her current plan.
- **Marketing Content** – Marketing activities and materials include or address content regarding any of the following:
 - Information about benefits or benefit structure
 - Information about premiums or cost-sharing
 - Rankings, such as comparisons to other Plan(s)/Part D sponsor(s)
 - Measurement standards, such as Star Ratings
 - Plan rewards and incentives programs (programs offered by Medicare health plans to qualifying individuals to voluntarily perform specified target activities in exchange for reward items)

16. What are Communication-only materials and activities?

Communication-only activities and material provide information to current and prospective enrollees that:

- contain no marketing content; or
- include marketing content but do not have marketing intent. For example,
 - CMS designates certain materials that have marketing content, such as the Evidence of Coverage, as a communication-only material, while the summary of benefits and an annual notice of change are designated by CMS as marketing materials.
 - A targeted letter sent to a plan’s members may be intended to encourage those members to receive certain preventive services they have not yet received and refer to the cost sharing for those services. While it contains marketing content, it would not meet the marketing intent standard.

17. Title Page – Regulation of Communications Activities and Materials

18. Regulation of Communications-Only Materials

Content -- While CMS does not generally require communications-only materials to be submitted to CMS for review and approval; it may review such materials and does require certain disclaimers to be used in those materials. CMS also prohibits the use of certain claims or language in communications materials.

- CMS does require certain designated communication materials critical to beneficiaries' understanding or accessing their benefits to be reviewed (e.g., the evidence of coverage).
- CMS may require prior review of communication materials that, based on feedback such as complaints or data gathered through reviews, warrant additional oversight to ensure accuracy.

19. Regulation of Communications-Only Activities

- **Contact** – CMS regulates how marketing representatives can contact potential enrollees. For example, agents may not make cold calls even if they limit their conversation to communications content.
- **Content** – CMS regulates what marketing representatives may say to enrollees and potential enrollees.

20. What are Examples of Communication Only Materials

- A flyer that says, "Healthy Choice Medicare plan has been serving beneficiaries for 30 years, call to find out if we are right for you!"
- A newsletter sent to members of Healthy Choice Medicare plan reminding them that preventive health care services have a \$0 copay and that it is important to get such services, such as flu shots, to stay healthy.
- Materials specifically designated by CMS as communications because they do not meet the "intent" standard, such as the Evidence of Coverage, formulary, or provider directory.

21. What are Communications Activities – Case Study

Agent Watson hands out flyers for his insurance agency at a senior expo. The brochures state that his agency can help seniors find the health plan that is right for them and lists the eight different Medicare Advantage organizations with which his agency contracts. The flyer also includes a telephone number that beneficiaries can call if they would like additional information. This activity is communication only and not marketing.

22. Regulation of Marketing Activities in General

CMS regulates marketing activities in a variety of ways, including:

- **Setting** – CMS has rules regarding marketing in a health care setting versus marketing in other settings and marketing at educational versus marketing events.

- **Who may market** – CMS requires marketing representatives to comply with state laws concerning licensure; CMS places strict limits on marketing by health care providers.
- **Timing** – CMS regulates when marketing representatives can begin marketing the next year's plans.
- **Contact**– CMS regulates how marketing representatives can contact potential enrollees.
- **Content** – CMS regulates what marketing representatives may say to enrollees and potential enrollees.

23. What are Examples of Marketing Activities?

Examples of marketing activities include:

- Providing a presentation on health plan benefits to Medicare beneficiaries where the intent is to steer them toward a plan or set of plans.
- Participating in an event where health plan brochures and a summary of benefits are distributed.
- Meeting with a Medicare beneficiary to discuss upcoming benefit enhancements by reviewing the Annual Notice of Change and to encourage him or her to remain enrolled in his or her current Medicare plan.
- Passing out plan-specific benefits information and agent business cards at a health fair.
- Accepting enrollment forms and performing enrollment at marketing/sales events.

24. What are Examples of Marketing Materials?

Examples of marketing materials include the following if they contain marketing content:

- General audience materials such as direct mailings, newspaper ads, or websites that promote specific plans and discuss their star ratings.
- Marketing representative scripts or outlines for telemarketing, enrollment, or other presentations that discuss plan benefits.
- Brochures that promote enrollment in a plan and discuss the plan's reward programs for receiving preventive care services.
- Presentation materials such as slides and charts explaining the benefits of enrolling in a particular plan.
- Social media posts (e.g., Facebook, Twitter, YouTube, etc.) that mention a plan's star rating, notes its low premium, or promote its benefits.

25. Regulation of Marketing Materials

CMS regulates materials that qualify as marketing materials as follows:

- **Content:** CMS may require that certain disclaimers or other information be included in marketing materials. CMS also prohibits the use of certain claims or language.
- **Review and Approval:** CMS requires that all marketing materials be submitted to CMS for approval and/or review.

- Materials developed for use with employer/union group members do not have to be submitted to CMS.

26. Rules that apply to all Communications (including Marketing) Materials and Activities

Plans and their marketing representatives may not:

- mislead, confuse, or provide materially inaccurate information to beneficiaries.
- target enrollees based on income levels (except in the case of Dual Eligible SNPs).
- target enrollees based on health status (except in the case of a chronic care SNP).
- state or imply that the plans are only available to seniors, rather than all eligible Medicare beneficiaries (e.g., younger disabled individuals).

27. Title Page- Marketing and Communications Prohibitions and Permissible Activities

28. Marketing and Communication Prohibitions - Unsolicited Contacts

Marketing representatives are prohibited from making unsolicited contact with beneficiaries, including through:

- door-to-door solicitation, including leaving leaflets, flyers, or door hangers at a residence or on someone's car.
- approaching beneficiaries in common areas such as parking lots, hallways, lobbies, sidewalks, stores, or parks.
- telephone calls.
- text messages and other forms of electronic direct messaging (e.g., through social media platforms, like Facebook instant messaging).
- voicemail messages.
- The prohibition on making unsolicited contact does not extend to e-mail, conventional mail, and other print media such as advertisements.

29. Unsolicited Contacts

- Marketing representatives may not make unsolicited calls about other business as a means of generating leads for Medicare plans (e.g., bait and switch strategies).
- Marketing representatives may not make unsolicited contact based on referrals provided by enrollees or other sources. However, they may leave business cards with beneficiaries for distribution to friends they are referring.
- Enrollees who are voluntarily disenrolling may not be contacted for sales purposes or be asked to consent to sales contacts.
- Marketing representatives may not make calls to beneficiaries who attended an event unless the beneficiary gave express permission at the event for a follow-up call (there must be documentation of permission to be contacted).

30. Unsolicited Contacts Example

Example 1: Agent Jackson sells life insurance in addition to MA plans. Agent Jackson calls a lead provided to him by another client to talk to her about life insurance. During the conversation, Agent Jackson asks the beneficiary if she would like to learn more about Medicare Advantage plans. Agent Jackson has violated the prohibition against making unsolicited calls about other business as a means of generating leads for Medicare plans.

Example 2: Agent Lopez's aunt tells him that her neighbor was recently telling her that she was having trouble understanding her Medicare choices and wanted to know if it made sense to enroll in an MA plan. His aunt offers the neighbor's telephone number, but agent Lopez gives his aunt his card and says that the neighbor should call him. Agent Lopez has wisely avoided making an unsolicited call based on a referral.

31. Permitted Contacts

Marketing representatives may:

- initiate electronic contact through e-mail. However, they must provide an opt-out process to no longer receive electronic communications.
- return calls or messages from individuals who initiate contact and request information.
- call beneficiaries who have expressly given permission for the contact, for example by filling out a business reply card or asking a plan customer service representative to have an agent contact them.
- call beneficiaries to confirm an appointment that has already been agreed to by a beneficiary.
- call beneficiaries who submit enrollment applications to conduct business related to enrollment.

32. Permitted Contacts, continued

Marketing representatives may:

- call current enrollees of the parent organization, including those in non-Medicare health plan products, to discuss plan business, for example, they may:
 - Contact individuals enrolled in one of the MA organization's commercial health plans when the individual is aging into Medicare
 - Contact the MA organization's Medicaid plan enrollees to discuss Medicare products
 - Contact current MA enrollees to promote other Medicare plan types or to discuss plan options/benefits
 - Contact the MA organization's Medigap enrollees regarding MA, PDP, or cost plan options.
- call current enrollees of a plan to discuss/inform them about general plan information such as Annual Enrollment Period dates, availability of flu shots, upcoming plan changes, educational events, and other important plan information.
- call a beneficiary who the marketing representative enrolled in a plan to discuss plan business, as well as discuss the availability of other plan options/types within the same parent organization.
- When reaching out to a beneficiary regarding plan business marketing representatives must offer the beneficiary the ability to opt out of future calls regarding plan business.

33. Title Page- Rules Related to Sales and Educational Events

34. Marketing and Educational Events

- Marketing/sales events are events designed to steer potential enrollees toward a plan or limited set of plans or to encourage current enrollees to remain in their plans.
- Educational events are events designed to inform potential enrollees about Medicare, including MA, Part D, or other Medicare programs and do not include marketing materials or activities.
 - Educational events may be held in public venues and hosted by the Plan/Part D Sponsor or an outside entity.
 - Educational events may be put on by providers or other groups and sponsored by one or more health plans.
- Sales representatives may conduct a marketing event immediately following an educational event in the same general location (e.g., the same hotel).
 - If a marketing event directly follows an educational event, beneficiaries must be made aware of the change and given the opportunity to leave before marketing begins.
- Advertisements and invitations (in any form of media) that are used to invite beneficiaries to a marketing or educational event must include the following statement: "For accommodation of persons with special needs at meetings call <insert phone and TTY number>."

35. Medicare Communications and Marketing Rules: Sales Events

- Plans must submit presentations that include marketing content to CMS before use at a sales event.
- At marketing/sales events agents may:
 - discuss plan-specific information such as premiums and benefits.
 - discuss plan star ratings.
 - discuss the merits of a plan
 - distribute and collect enrollment applications.
 - distribute plan-specific advertisements, explanatory information, and general information about Medicare.
 - provide refreshments and light snacks of nominal value as long as they are not bundled and provided as if a meal.

36. Sales Events, Prohibited Activity

At marketing/sales events agents may not:

- require beneficiaries to provide contact information as a prerequisite for attending the event.
 - This includes requiring an email address or other contact information as a condition to RSVP for an event online or through the mail.
 - Sign-in sheets must be labeled as optional.
- conduct health screenings or other activities that could give the impression of "cherry-picking."

- provide meals or multiple snacks/refreshments bundled and provided as if a meal, regardless of value.
- use information collected for raffles and drawings for other purposes.

37. Light Snacks Versus Meals

Example 1: Agent Foley puts out pretzels, chips, cookies, water, and soda during her marketing seminar to promote HealthyChoice Medicare plan. Such food offerings qualify as light snacks, which may be offered at marketing events.

Example 2: To promote an early morning marketing seminar, Super Agency provides pastries, muffins, bagels, cream cheese, yogurt, fruit, juice, and coffee. Super Agency has bundled several items that might have been considered snacks as a continental breakfast (i.e., a meal), which is prohibited at a marketing event, but allowable at an educational event if the value does not exceed \$15 per person (nominal value).

38. Educational Events

Educational events must be explicitly advertised as “educational.” At educational events, marketing representatives may:

- engage in communications activities and distribute communication materials.
- use a banner with the plan sponsor name and/or logo displayed.
- distribute promotional items, including those with the plan name, logo, and toll-free number and/or website. These items must be free of marketing content and consistent with nominal gift rules.
- provide an objective presentation to educate beneficiaries about the different ways they can get their Medicare benefits.
- have a health care provider make an educational presentation on wellness or another health care related topic.

39. Educational Events, Permitted Activities

At educational events, marketing representatives may:

- answer beneficiary-initiated questions about Medicare health or drug plans.
- set up future personal marketing appointments.
- obtain beneficiary contact information including completed Scope of Appointment (SOA) forms).
- distribute business cards and agent/agency or plan contact information for beneficiaries to initiate contact.
- provide meals, refreshments, or snacks as long as they comply with the nominal gift requirements.

40. Educational Events, Impermissible Activities

When an event has been advertised as “educational,” marketing representatives may NOT:

- conduct sales or marketing presentations.

- discuss, display, or distribute plan-specific premiums, benefits, or marketing materials.
- engage in marketing activities.
- distribute or collect enrollment applications.

41. Gifts and Promotional Items

Section 1128A (a) (5) of the Social Security Act prohibits offering or giving anything of value to a Medicare or Medicaid beneficiary that is likely to influence the beneficiary to order or receive from a particular provider, practitioner, or supplier any item or service covered under Medicare or Medicaid. There is a nominal value (\$15) exception to this rule.

- Marketing representatives may provide gifts, prizes, or promotional items to beneficiaries as part of an event or for marketing purposes as long as the nominal value exception is met and the gift is provided regardless of enrollment and without discrimination.
- Gifts are of nominal value if an individual item is worth \$15 or less based on the retail purchase price of the item (it does not matter if the plan or representative pays less for the item).
- When more than one gift is offered on one occasion, the combined value of all items must not exceed \$15.
- Multiple gifts given to a beneficiary on different occasions may not exceed \$75 aggregate, per person, per year.

42. Gifts and Promotional Items, Continued

Gifts or prizes must not be in the form of cash or cash equivalents or other monetary reward or rebate even if their worth is less than \$15.

- Cash equivalents include:
 - gift certificates or cards that can be readily converted to cash.
 - general gift cards that are not restricted to specific retail chains or to specific items and categories, such as VISA gift cards.
 - gift cards for retailers or online vendors that sell a wide variety of consumer products (e.g., Walmart and Amazon).
 - debit cards.
- A gift card that can be used for a more limited selection of items or food, would not be considered a cash equivalent (e.g., Starbucks or a Shell Gas gift card).
- Rebates would include, for example, a discount on the first month's premium or on a copayment. Gifts or prizes may not be charitable contributions.

43. Promotional Activities: Drawings, Prizes, Giveaways

- Plan sponsors must include a disclaimer on all marketing materials promoting a prize. drawing, communal experience (e.g., a concert), or any promise of a gift that there is no obligation to enroll in the plan.

44. Title Page –Marketing at Individual Appointments

45. Individual Marketing Appointments

- Personal/Individual marketing appointments are those tailored to an individual or small group (e.g., husband and wife). They are not defined by the location.
- During a personal/individual appointment, representatives may market only health care related products identified in a scope of appointment.
 - Such products include Medicare health plans, Medigap plans, and dental plans, but not accident-only plans.
 - Non-health care products may not be marketed during a Medicare health or drug plan appointment.

46. Required Practices: Scope of Appointment

- Before any marketing appointment, marketing representatives must coordinate with the beneficiary to identify the types of product(s) that will be discussed, obtain agreement from the beneficiary, and document that agreement (known as a “scope of appointment”).
 - Types of products include MA, PDP, Cost plans, and Medicare-Medicaid Plans.
 - This includes when a beneficiary unexpectedly initiates contact, for example, by unexpectedly calling or walking into a marketing representative's office, or unexpectedly attending a sales appointment properly set up for another individual.
- A new SOA is required if the beneficiary requests information regarding a different plan type than previously agreed upon.

47. Required Practices: Scope of Appointment, continued

- A scope of appointment may be in writing, in the form of a signed agreement (including electronic signature) by the beneficiary, or a recorded oral agreement. Any technology (e.g., conference calls, fax machines, designated recording lines, pre-paid envelopes, and email) can be used to document the scope of appointment.
- For appointments made over the phone, documentation is generally a recording of the call. However, if the agent does not have a recording mechanism, the agent must obtain a written agreement signed by the beneficiary or authorized representative. This can happen immediately before conducting the appointment.
- A plan sponsor or agent may not agree to the scope of appointment on behalf of the beneficiary.

48. Scope of Appointment: Documentation

Plans/Part D Sponsors are expected to include the following when documenting the SOA:

- Product type (e.g., MA, PDP) that the beneficiary has agreed to discuss during the appointment,
- Date of appointment,
- Beneficiary and agent contact information (e.g., name, address, telephone number),
- Written or verbal documentation of beneficiary or appointed/authorized representative agreement,

- A statement that beneficiaries are not obligated to enroll in a plan; their current or future Medicare enrollment status will not be impacted and clearly explaining that the beneficiary will not be automatically enrolled in any plan.

49. Individual Marketing Appointments, Allowed Activities

During individual appointments, marketing representatives may:

- distribute plan materials such as an enrollment kit or marketing materials;
- provide educational information;
- discuss benefits, premiums, and cost sharing;
- talk about Plan rewards and incentives programs; and
- provide and collect enrollment forms.

50. Individual Marketing Appointments, Prohibited Activities

During individual appointments, marketing representatives may not:

- discuss plan options that were not agreed to in the scope of appointment before the appointment began, unless during the appointment, the beneficiary requests information on a plan type other than agreed upon and a new scope of appointment is completed;
- market non-health care related products; or
- solicit/accept an enrollment request for a January 1st effective date before the start of the Annual Election Period on October 15 unless the beneficiary is entitled to another enrollment period (for example, an initial enrollment period or special enrollment period).

51. Use of Social Media to Market

- Plans/Part D Sponsors must submit to CMS social media (e.g., Facebook, Twitter, YouTube, LinkedIn, Scan Code, or QR Code) posts that meet the definition of marketing materials, specifically those that contain marketing content such as plan-specific benefits, premiums, cost-sharing, or Star Ratings. This includes such posts by agents.
- Social media posts are generally subject to marketing/communications content requirements, such as prohibitions on using certain language.
- Re-publication (or re-post) of an individual's post, content or comment that promotes a Plan's/Part D Sponsor's product from social media sites (e.g., Facebook, Twitter, YouTube, LinkedIn, Scan Code, or QR Code) is considered a product endorsement/testimonial and must adhere to the guidance on testimonials.

52. Social Media Example

Agent LaVette is excited about the 2023 Medicare Advantage plan that will be offered by BetterMedicare. She would like to send out a tweet stating, "Call me to learn more about BetterMedicare's Medicare Advantage Plan offering comprehensive eyecare and dental benefits at only \$25 per month." Because it contains marketing content, this tweet must be submitted to CMS. However, Agent LaVette could send out a tweet stating "BetterMedicare offers an array of benefit

packages. One might be right for you. Call me to find out more!” as long as her contract with BetterMedicare does not prohibit her from tweeting such communications.

53. Title Page-Required Practices

54. Required Practices: Marketing Activities

Marketing representatives must:

- provide to prospective enrollees only CMS-approved/submitted marketing materials or CMS created marketing materials.
- use only CMS-approved/submitted talking points and presentations if they are marketing.

55. Required Practices: HIPAA and Confidentiality of Enrollee Information

Marketing representatives are business associates of the health plans they sell. As such, they must comply with Federal and state law regarding the confidentiality of individually identifiable health information (known as protected health information or PHI) and any confidentiality obligations in their business associate contracts with plans.

- PHI includes any information about an individual’s health care coverage, payment for health care, or health care condition.
- HIPAA regulates both the use and disclosure of PHI. Marketing representatives may only use and disclose PHI on behalf of the plans they represent.
- De-identification is the use of PHI. Thus, agents cannot de-identify PHI in order to use it for their own purposes.
- If a marketing representative subcontracts with another individual or entity that may obtain, use or disclose PHI on their behalf, they must enter into a business associate agreement with the individual or entity requiring the same protections to the PHI.

56. Required Practices: Marketing & Non-Health Related Activities

- Plans/Part D Sponsors must obtain a HIPAA compliant authorization from an enrollee before the Plan or Part D sponsor may use (or may request a marketing representative to use on their behalf) information about the enrollee to market non-health related items or services such as accident-only policies, life insurance policies, or annuities.
- The HIPAA authorization must specify that the individual’s information may be used by the Plan/Part D sponsor for marketing purposes.

57. Required Practices: Use of TPMO Marketing Disclaimer

The following requirements apply to the marketing of plans for 2023 and the subsequent years, which will begin on October 1, 2022:

- Independent agents/brokers and other third party marketing organizations must use the TPMO marketing disclaimer unless the TPMO sells all commercially available MA/Part D plans in a given service area.
The disclaimer consists of the statement: “We do not offer every plan available in your area. Any information we provide is limited to those plans we do offer in your area. Please contact Medicare.gov or 1–800–MEDICARE to get information on all of your options.”
- The disclaimer must be:
 - a. verbally conveyed within the first minute of a sales call;
 - b. electronically conveyed when communicating with a beneficiary through email, online chat, or other electronic means of communication;
 - c. prominently displayed on TPMO websites; and
 - d. included in any TPMO marketing materials, including print materials and television advertising.

58. Title Page- Accessing and Using Certain Plan Materials

59. Accessing and Using Certain Plan Materials

Plans are required to post on their website certain documents in a downloadable format. Beneficiaries and marketing representatives may access the following materials, as applicable to the plan, through each plan’s website:

- Evidence of Coverage
- Summary of Benefits
- Annual Notice of Change
- Provider Directory
- Pharmacy Directory
- Formulary

Such documents are important tools to be used by marketing representatives in determining issues such as:

- Whether a beneficiary’s providers or pharmacies are in the plan network
- Whether the drugs a beneficiary takes are on the plan’s formulary
- Whether the plan covers other benefits that are important to a beneficiary
- Whether or the extent to which the plan covers out-of-network services

Many plans provide their provider/pharmacy directories and formularies in a searchable format. Plans will also provide these materials to beneficiaries after they enroll.

60. Required Materials with an Enrollment Form

- When a beneficiary is provided with enrollment instructions/form, they must also receive:
 - plan ratings information;
 - the summary of benefits; and
 - the pre-enrollment checklist
 - Applicable for 2023 and subsequent year plans, the beneficiary must also receive the multi-language insert when provided with an enrollment form or any other CMS required material (for example, Evidence of Coverage, Annual Notice of Change, or Summary of Benefits).
- For Dual Eligible Special Needs Plans, if Medicaid benefits are not included in the summary of benefits, a separate document including the Medicaid benefits must be included with the enrollment form.
- When a beneficiary enrolls in a plan online, the plan sponsor must make these materials available electronically, (e.g., via website links) to the potential enrollee before the completion and submission of the enrollment request.
- For telephonic enrollments, the beneficiary must be told verbally where the summary of benefits and star ratings documents may be accessed.

61. Title Page-Plan Ratings

62. Plan Ratings: Background

- CMS releases star ratings that allow beneficiaries to compare MA plans and Part D plans. These ratings include topics such whether members got various screening tests, vaccines, and other check-ups to help them stay healthy, and how members rate the plan on topics such as access to care and customer service.
- Marketing representatives and beneficiaries who have access to the Internet may obtain plan rating information at <http://www.medicare.gov>.
 - Click the “Find Health & Drug Plans” button on the left.
- The Star Ratings information document must also be prominently posted on each plan’s website.

63. Plan Ratings – Required Practices

- Plan sponsors must provide the plan’s overall performance ratings to beneficiaries in the standard Plan Ratings information document.
- New Plans/Part D Sponsors that do not have any Star Ratings information are not required to provide Star Ratings information until the next contract year.
- Plan sponsors and their marketing representatives may only reference or mention a plan’s rating on an individual measure in conjunction with the plan’s overall performance rating (MA-PD), the contract’s highest rating, Part C summary rating (MA-only), or Part D summary rating (PDPs), with equal or greater prominence.
- Plan sponsors and their marketing representatives may only market the Star Ratings in the service area in which the Star Rating is applicable.

64. Required Practices: Plan Ratings - Prohibitions

Plan sponsors and their marketing representatives may not:

- use a plan's star rating in an individual category or measure to imply a higher overall plan rating than is actually the case.
 - For example, a plan which received a 5-star rating in customer service promotes itself as "rated 5-stars by our enrollees," when its overall plan rating is only 4 -stars.
- use the plan's star ratings in a manner that misleads beneficiaries into enrolling in plans based on inaccurate information.
- use updated star ratings until CMS releases star ratings on the Medicare Plan Finder.
- continue to use an old star rating after 21 days from the release of a new star rating.

65. Required Practices: Recording Calls with Beneficiaries

Beginning October 1, 2022, independent agents/brokers and other third-party marketing organizations must record all calls with beneficiaries in their entirety, including the enrollment process when marketing plans for 2023 and subsequent years.

66. Title Page- Prohibited Practices and Marketing Prohibitions

67. Prohibited Practices: Marketing and Communications (#1/6)

Marketing representatives must NOT:

- market any non-healthcare related products (such as annuities and life insurance) during any MA or Part D sales activity or any other marketing activity for existing enrollees. This is considered cross-selling; or
- use or disclose the enrollee's protected health information (PHI) for marketing purposes, including sending any non-plan or non-health related information or otherwise contacting him/her for purposes unrelated to plan benefits administration or CMS contract execution, without first obtaining HIPAA required authorization from the enrollee.

68. Prohibited Practices: Solicit Applications(s) Before October 15 (#2/6)

Marketing representatives must NOT:

- use a Medicare beneficiary to endorse a plan unless the beneficiary was an enrollee of the plan when the endorsement was created.
- solicit enrollment applications for the following contract year before the start of the annual election period on October 15.
- use marketing materials that have not been submitted by the plan for review and/or approval by CMS.
- charge beneficiaries marketing fees.

- Engage in bait and switch strategies such as making unsolicited outbound calls to beneficiaries about other lines of business (e.g., calling Medicare beneficiaries about Affordable Care Act plans) as a means of generating leads for Medicare plans.

69. Prohibited Practices: Discriminatory Activity (#3/6)

Marketing representatives must NOT:

- engage in any discriminatory activity such as attempting to recruit Medicare beneficiaries from higher-income areas without making comparable efforts to enroll Medicare beneficiaries from lower-income areas;
- encourage individuals to enroll based on their health status unless the plan is a special needs plan that focuses on the beneficiary's particular condition;
- conduct health screening or other activities that could give an impression of "cherry-picking";
- assert that their plan is the "best" plan or use other unsubstantiated absolute or qualified superlatives or pejoratives. (However, unsubstantiated absolute and/or qualified superlatives may be used in logos/taglines); or
- make explicit comparisons between plans, unless they can support them, such comparisons are factually based, and the comparisons are not misleading.

70. Prohibited Practices: Misleading Marketing Practices (#4/6)

Marketing representatives must NOT engage in marketing practices that may mislead or confuse beneficiaries, such as:

- engaging in scare tactics. This would include activities such as pressuring a hesitant beneficiary to decide in a very short period of time by misleading them to believe they would not have any health benefits if they did not enroll in a plan;
- providing false or misleading information about the plan, including about benefits, provider rules, and other plan information, such as claiming that a PFFS plan is the same as Original Medicare or a Medigap plan;
- claiming that Medicare, CMS, or any government agency endorses or recommends the plan;
- Stating that they are from Medicare or using words or symbols, including "Medicare" in a misleading manner. For example, Agents cannot state that they are approved, endorsed or authorized by Medicare, are calling on behalf of Medicare, or that Medicare asked them to call or see the beneficiary.

71. Prohibited Practices: Use of Term "Free" (#5/6)

Marketing representatives must NOT:

- use the term "free" to describe a zero-dollar premium;
- use the term "free" in conjunction with any reduction in premiums, deductibles, or cost-share, including Part B premium buy-down, low-income subsidy, or dual eligibility;

- lead beneficiaries to believe that the broker or agent works for Medicare, CMS, or any government agency;
- imply that an MSA plan operates as a supplement to Medicare;
- market that the Plan Sponsor will not disenroll individuals due to failure to pay premiums.

72. Prohibited Practices: D-SNP Cautions (#6/6)

Unless they are promoting a D-SNP, Marketing representatives must NOT:

- imply that the plan is available only to or designed for dual eligible individuals ((unless the plan is comparable to a D-SNP plan, such as an MMP, as determined by the Secretary));
- claim that the Plan has a relationship with the state Medicaid agency, unless the MA plan (or its parent organization) has contracted with the state to coordinate Medicaid services, and the contract is specific to that MA plan (not for a separate D-SNP or MMP);
- market a non-dual eligible special needs plan as if it were a dual-eligible special needs plan; or
- target their marketing efforts for the Plan exclusively to dual eligible individuals (unless the plan is comparable to a D-SNP plan, such as an MMP, as determined by the Secretary).

73. Open Enrollment Period – Marketing Prohibitions

The Medicare Advantage Open Enrollment Period (MA-OEP) is a period during which an individual enrolled in an MA or MA-PD plan can make a one-time change to another MA plan, elect Original Medicare, or can change Part D coverage. For individuals enrolled in an MA plan on January 1, the MA-OEP is the first 3 months of the calendar year (different rules apply to individuals choosing an MA plan during their initial coverage election coverage period (ICEP)). The MA-OEP is further described in Module 5.

During the Open Enrollment Period (OEP), marketing representatives may **not**:

- send unsolicited materials advertising the ability/opportunity to make an additional enrollment change or referencing the OEP.
- specifically target beneficiaries who are in the OEP because they chose an MA or MA-PD plan during the Annual Enrollment Period (AEP) by the purchase of mailing lists or other means of identification.
- engage in or promote activities that intend to target the OEP as an opportunity to make further sales.
- call or otherwise contact former enrollees who have selected a new plan during the AEP.

74. Promoting Health Plans During the Open Enrollment Period

However, during the OEP, marketing representatives may conduct marketing activities that focus on other enrollment opportunities, including but not limited to:

- marketing to individuals turning 65 (who have not yet made an enrollment decision).
- marketing by 5-star plans regarding their continuous enrollment SEP.

- marketing to dual-eligible and LIS beneficiaries who, in general, may make changes once per calendar quarter during the first nine months of the year.

In addition, during the OEP marketing representatives may:

- send marketing materials when a beneficiary makes a proactive request.
- at the beneficiary's request, have one-on-one meetings with a sales agent.
- at the beneficiary's request, provide information on the OEP through the Plan's call center.

MAOs may also include information about the OEP on their websites.

75. Soliciting Referrals from Beneficiaries

In the past, the Medicare Communications and Marketing Guidelines (MCMG) contained rules around soliciting referrals from beneficiaries, including current enrollees. Those rules limited when a marketing representative could solicit such referrals and what types of gifts (nominal value) marketing representatives could provide in exchange for a referral. CMS has removed these provisions from the MCMG. Inducements (such as gifts) for referrals may still be regulated under fraud and abuse laws (such as the anti-kickback statute or beneficiary inducement statute). However, typically there are nominal value exceptions under those laws. Marketing representatives should consult with their plans to determine whether the plans impose requirements around beneficiary referrals.

76. Title Page-Marketing, Oversight, and Enforcement of Marketing Requirements

77. Marketing Activities: Marketing in a Health Care Setting

Marketing representatives must NOT:

- engage in marketing activities in areas where patients receive health care services, for example:
 - Exam rooms, dialysis center treatment areas, or hospital patient rooms, pharmacy counter areas, and other treatment areas where patients interact with a provider and his/her clinical team and receive treatment.

Marketing representatives may:

- engage in marketing activities (i.e., conduct sales presentations and distribute and accept enrollment applications) in common areas of health care settings, for example:
 - In a cafeteria, community or recreational room, waiting room, common entryway, vestibule, or conference room
 - At a retail pharmacy, in areas away from the pharmacy counter
- provide communication materials to be distributed and displayed in the healthcare setting.

78. Marketing Activities: Marketing in a Long-term Care Facility

- Long-term care facilities include, for example, nursing homes, assisted living facilities, and board and care homes.

- Plan sponsors/marketing representatives may schedule an appointment with a beneficiary in a long-term care facility ONLY upon request of the beneficiary (or authorized representative).
- Plan sponsors/marketing representatives may not visit individuals in a long-term care facility without an appointment.
- MA institutional special needs plans (I-SNPs) may use staff operating in a social worker capacity to provide information, including marketing materials concerning I-SNPs to residents. Such information must not include an enrollment form and the social worker may not accept or collect a scope of appointment or enrollment form on behalf of the plan sponsor.
- Marketing representatives may set up in common areas of a long-term care facility and allow residents to approach them.

79. Marketing to Employer/Union Groups

Marketing representatives and Plans must follow all marketing rules and guidelines when marketing employer group health plans except the following:

- the prohibition against unsolicited contacts
- the prohibition against cross-selling other products
- the requirement to obtain prior documentation of the scope of an appointment
- the prohibition against providing meals at marketing events
- the pre-enrollment checklist requirement
- marketing representative compensation requirements
- the requirement that a marketing representative must pass an annual test, although the requirement for annual training does apply

Plans offering employer group health plans are not required to submit marketing materials specific only to those employer plans to CMS at the time of use. However, CMS may request and review copies if employee complaints occur or for any other reason to ensure the information accurately and adequately informs beneficiaries about their rights and obligations under the plan.

80. Plan Oversight and Enforcement

- Plan Sponsors are responsible for ensuring compliance with CMS' current marketing regulations and guidance, including monitoring and overseeing the activities of their third party marketing organizations, marketing representatives, and providers.
- Plans are required to implement a strategy to prevent prohibited marketing practices from occurring, detect prohibited marketing tactics at their early stages, and take immediate corrective action to respond to non-compliant marketing activities.
- Plans must take disciplinary and/or corrective action in the event of verified misconduct. Examples of such disciplinary action include:
 - Withholding or withdrawing commissions
 - Retraining
 - Suspension of marketing
 - Termination

81. Plan Oversight and Enforcement, continued

- Plans must comply with requests from a State insurance or other department in connection with investigations of plan marketing representatives who are licensed by the department.
- Plans must report to States the termination of any agent or broker, including the reasons for the termination if required under State law. Plans must also report to CMS any for-cause agent/broker terminations.
- Report to CMS all enrollments made by unlicensed agents or brokers.
- Upon CMS' request, the plan must provide CMS with information necessary for it to conduct oversight of marketing activities.

82. Plan Oversight and Enforcement: New for Marketing Plan Years 2023 and Beyond

Plan sponsors must require third party marketing organizations, including independent agents and brokers, to:

- Disclose to the plan sponsor any subcontracted relationships used for marketing, lead generation, and enrollment.
- Report to plan sponsors monthly any staff disciplinary actions or violations of any requirements that apply to the plan sponsor associated with beneficiary interaction to the plan.

83. Special Rules Related to Lead Generation

The following requirements apply to the marketing of plans for 2023 and subsequent years, which will begin October 1, 2022.

- TPMOs conducting lead generating activities, either directly or indirectly for a plan sponsor, must, when applicable disclose to the beneficiary that his or her information will be provided to a licensed agent for future contact. This disclosure must be provided:
 - verbally when communicating with a beneficiary through telephone;
 - in writing when communicating with a beneficiary through mail or other paper; or
 - electronically when communicating with a beneficiary through email, online chat, or other electronic messaging platform.
- If the lead generating TPMO transfers a beneficiary to an agent, the TPMO must disclose to the beneficiary that he or she is being transferred to a licensed agent who can enroll him or her into a new plan.
- Plan Sponsors, Agents/Brokers, FMOs or agencies that obtain leads from such entities should ensure that these requirements have been met in generating the leads and include this obligation in any contracts with the lead generating TPMO.

84. Title Page-Marketing Representative Compensation

85. Marketing Representative Compensation: Compensation Defined

Compensation includes monetary or non-monetary compensation of any kind relating to the sale or renewal of a policy including, but not limited to:

- Commissions
- Bonuses
- Gifts
- Prizes or awards

Compensation does not include:

- payment of fees to comply with State appointment laws, training, certification, and testing costs.
- reimbursement for mileage for appointments with beneficiaries
- reimbursement for placing ads or purchasing flyers
- actual costs associated with beneficiary sales appointments such as venue rent, snacks, and materials.

86. Marketing Representative Compensation: Applicability of Rules

- Compensation structures for independent agents must comply with CMS guidance.
 - Compensation rules do not generally apply to marketing representatives who are plan employees, to “captive” agents who market for only one plan, or when independent agents are marketing only to employer/union groups.
 - Employed and captive agents/brokers who only sell for one Plan are subject to the requirements related to referral/finder fees.
 - Compensation to independent agents who market to and enroll beneficiaries is covered by the rules whether it is paid directly by a plan or paid by an agency, Field Marketing Organization (FMO), Third-Party Marketing Organization (TMO), or similar organization.

87. Marketing Representative Compensation: When Compensation May Not Be Paid

Plan Sponsors may not pay agents:

- when they have not been trained and tested.
- when they do not meet state licensure/appointment requirements.
- When a Plan Sponsor and/or a contracted independent agent terminates an agent contract, any future payment for an existing business will be governed by the terms of the contract that specifies the agent’s payment, subject to the limits in the CMS regulation.
- However, to continue receiving renewal fees, agents must remain trained, tested, licensed, and appointed (to the extent required under state law), regardless of whether they are actively selling.

88. Marketing Representative Compensation: Applicable Amounts

- Applicable compensation amounts depend on whether enrollment is an initial year enrollment or a renewal year enrollment.
 - CMS provides reports to the plan that specifies whether enrollment is initial or renewal.
- Renewal year enrollments include plan changes between “like plans.”
 - A “like plan type” enrollment includes:
 - A PDP to another PDP
 - An MA, MA-PD, or MMP to another MA, MA-PD, or MMP
 - A Section 1876 Cost Plan to another Section 1876 Cost Plan
 - An “unlike plan type” enrollment includes:
 - An MA or MA-PD plan to a PDP or Section 1876 Cost Plan
 - A PDP to a Section 1876 Cost Plan or an MA (or MA-PD) plan
 - A Section 1876 Cost Plan to an MA (or MA-PD) plan or PDP

89. Marketing Representative Compensation: Applicable Amounts, continued

- For enrollments in two plans at once (for example, enrollment in an MA-only plan like a MSA and a stand-alone PDP or a cost plan and a PDP), the compensation rules apply independently to each plan. This is known as “dual plan” enrollment.
- However, when dual plans are replaced by an enrollment in a single plan, compensation is paid based on the MA or cost plan movement (e.g., movement from an MA-only PFFS plan and PDP to an MA-PD plan would be compensated at the renewal compensation amount for the MA to MA-PD “like plan type” move).

90. Marketing Representative Compensation: Limits on Amount of Compensation and Filings

- Compensation for initial year enrollments cannot exceed a fair market value (FMV) published annually by CMS. This amount is known as the FMV cut-off.
- Compensation for renewal year enrollments cannot exceed 50 percent of the FMV cut-off. However, it can be less than 50 percent or not paid at all.
- Referral or finders’ fees paid to independent, captive or employed agents/brokers may not exceed \$25 for PDPs or \$100 for all other types of plans.
- Plans must annually file with CMS whether they will use independent agents/brokers and, if so, the amounts or range of amounts that they intend to pay for enrollment commissions.
- Once the deadline for filing has passed, plans may not change the amounts they pay or the types of brokers they use.

91. Compensation Example

HealthyChoicePlus Medicare Advantage plan pays agents the maximum allowable amounts for enrollments. HealthyChoicePlus wants to incentivize agents to sell more HealthyChoicePlus plans, so it

wishes to offer a \$200 bonus to its Agents for every 10 new individuals they enroll in HealthyChoicePlus. HealthyChoicePlus appropriately abandons this plan when it realizes the bonus would be considered compensation and total compensation would exceed the FMV cut-off.

92. Marketing Representative Compensation: Rules

- Compensation is paid on a calendar year basis. Thus, regardless of the month of a beneficiary's initial year enrollment, the renewal year begins on January 1 of the subsequent year, NOT on the beneficiary's enrollment anniversary date.
- Plan Sponsors have the flexibility to make compensation payments annually, quarterly, monthly, or by a different schedule. However, Plans/Part D Sponsors must pay compensation payments during the year of enrollment.
- Compensation (with some limited exceptions) may only be paid for the months the beneficiary is enrolled in the plan.
 - If a plan pays compensation in advance, it must recoup amounts paid for months a beneficiary is not enrolled.
 - If a beneficiary enrolls mid-year, compensation must be prorated.

93. Marketing Representative Compensation: Exception to Pro Rata Payment Rule

- A plan may choose to pay for an entire initial enrollment year, despite less than 12 months of enrollment, for a beneficiary who has never been enrolled in a plan before or where a beneficiary moves from an employer group plan to a non-employer group plan.
- However, if the plan pays a full initial compensation and the enrollee disenrolls during the contract year, the plan must recoup a pro-rated amount for all months the enrollee is not enrolled.

Example: Ms. Franklin turns 65 in August and is eligible for Medicare for the first time. An agent helped her enroll in an MAPD with a September 1 effective date. The plan will pay the agent at the initial year compensation level. The plan may choose to pay for the entire year at the initial rate if Ms. Franklin remains enrolled for the rest of the calendar year or for only September through December (4 months) at the initial rate. The Plan's payment obligation will be outlined in the Plan's contract with the agent or agent's brokerage firm (as applicable).

94. Marketing Representative Compensation: Rapid Disenrollment

- If a beneficiary disenrolls within the first 3 months of enrollment (referred to as "rapid disenrollment"), the entire compensation amount paid for the enrollment must be recouped, except under certain circumstances.
 - plans do not have to recoup any compensation paid including when a beneficiary disenrolls in the first 3 months because the beneficiary:
 - Enrolls effective October 1, November 1, or December 1 and subsequently uses the Annual Election Period to change plans for an effective date of January 1.
 - Became dually eligible for both Medicare and Medicaid
 - Qualified for another plan based on special needs
 - Became LIS eligible
 - Loss Medicare entitlement

- Moved out of the service area
- Failed to pay the plan premium
- Changed enrollment to a plan with a 5-star rating or disenrolled from a LPI plan to move into a plan with three or more stars
- Moved into or out of an institution
- Gained/dropped employer/union sponsored coverage
- Changed plans due to an auto, facilitated, or passive enrollment
- Died.
- The plan terminated, non-renewed, or CMS imposed sanctions on the plan.

95. Marketing Representative Compensation: Rapid Disenrollment, continued

- Rapid disenrollment applies when an enrollee moves from one parent organization to another parent organization, or when an enrollee moves from one plan to another plan within the same parent organization.

Example of rapid disenrollment: An Agent assisted Ms. Howard in enrolling in a Medicare Advantage HMO plan during the Annual Enrollment Period. After enrolling, she realized her podiatrist was not in the network. In February she switched to a PPO offered by the same organization so that she would have coverage for out-of-network services. The plan must recoup all compensation payments paid to the Agent for Ms. Howard's enrollment.

96. Title Page-Frequently Asked Questions

97. Frequently Asked Questions (FAQ) #1

Q: We purchased books on health maintenance that we plan to give away to anyone attending one of our marketing presentations, regardless of whether they enroll in our plan. Because we purchased many books, we were able to buy them for \$14.99 per book. However, on the inside jacket, the retail price is shown as \$19.99. May we give these books away at our marketing presentation?

A: No. The retail purchase price of the book is \$19.99, which exceeds CMS's definition of nominal value.

98. Frequently Asked Questions (FAQ) #2

Q: We would like to offer gifts of nominal value to potential enrollees who call for more information about our plan. We would then like to offer additional gifts if they come to a separate marketing event. Each of the gifts meets CMS's definition of nominal value, but taken together, the gifts are more than the nominal value. Is this permissible?

A: Yes. A nominal value gift may be given to an enrollee on more than one occasion if the total of all nominal value gifts given to an enrollee in a year does not exceed \$75.

99. Frequently Asked Questions (FAQ) #3

Q: Listed below are some possible promotional items to encourage potential enrollees to attend marketing presentations. Are these types of promotions permissible?

- Light Snacks (no meals)
- Day trips
- Magazine subscriptions
- Event tickets

A: Yes. All these promotional items are permissible if they are offered to everyone who attends the event, regardless of whether they enroll and if the gifts have a retail value of \$15 or less per marketing event. Cash gifts and monetary equivalents are prohibited, including gift certificates and gift cards that can be readily converted to cash, regardless of the dollar amount.

100. Frequently Asked Questions (FAQ) #4

Q: Can a marketing representative hold a raffle or offer a door prize or a drawing for a prize of more than nominal value for those who attend a marketing presentation if the total value of the item is less than \$15 per person attending?

A: Yes. However, the total fair market value must not exceed the nominal per person value based on attendance. For example, if 10 people are expected to attend an event, the nominal gift may not be worth more than \$150 (\$15 for each of the 10 anticipated attendees). For planning purposes, anticipated attendance may be used, but must be based on actual venue size, response rate, or advertisement circulation.

101. Frequently Asked Questions (FAQ) #5

Q: Can a marketing representative give away amazon gift certificates worth \$10 to everyone who attends a marketing presentation?

A: No. Cash gifts and monetary equivalents are prohibited, including gift certificates and gift cards that can be readily converted to cash, regardless of the dollar amount.

102. For More Information

- Medicare Marketing Guidelines: <http://www.cms.gov/Medicare/Health-Plans/ManagedCareMarketing/FinalPartCMarketingGuidelines.html>
- CMS Marketing Website: <http://www.cms.gov/ManagedCareMarketing/>
- Medicare Beneficiary Website: www.medicare.gov